

Policy on Availability and Retention of Current Documents, Records, Data and Information

«Hospital Name»

Document control

Document No./Version	_____ / 1.0	Issue No./Review due	01 / One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Medical Records In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» ensures the availability of current and relevant documents, records, data and information, and provides for their retention and eventual destruction in an orderly, compliant manner.

1. Purpose

This policy sets out how «Hospital Name» controls documents so only current, authorised versions are in use; retains patients' clinical records, data and information per its requirements; keeps the retention process confidential and secure; and destroys records, data and information per written guidance.

This policy does not cover medical record content or confidentiality/security of active records — those are covered in other hospital policies.

2. Scope

This policy applies to all documents, records, data and information — physical and electronic — held by «Hospital Name», from creation through retention to destruction.

3. Policy standards

«Hospital Name» has an effective process for document control.

«Hospital Name» retains patients' clinical records, data and information according to its requirements.

The retention process at «Hospital Name» provides expected confidentiality and security.

The destruction of medical records, data and information at «Hospital Name» is in accordance with written guidance.

4. Non-negotiable rules

1. Do not use a document — form, format, policy or procedure — that is not current, relevant, and reviewed, authorised and released by a designated individual.
2. Do not leave an obsolete document at the site of use — it must be removed and archived per the planned retention period.
3. Do not set a retention period for out-patient, in-patient or medico-legal case records outside what is consonant with National Medical Commission rules and the respective state authority.

4. Do not let the retention process compromise confidentiality or security — the same expectations apply to manual and electronic systems.
5. Do not destroy a medical record, data or information without approval from the concerned internal/external authority and without following relevant statutory provisions.
6. Staff who see a rule under this policy broken report it the same shift to the Medical Records In-Charge or the Medical Superintendent.

5. What we do

5.1 Document control

«Hospital Name» ensures all documents in use — forms, formats, policies and procedures — are current and relevant. Documents are created, reviewed for adequacy, authorised and released by designated individuals, and reviewed for updating per a planned schedule. All approved documents are identifiable. Obsolete documents are removed from the site of use and archived per a planned retention period, based on the organisation's policy. Only current valid versions are available for use. Required policies, procedures and plans are available, and staff understand how to access those relevant to their work.

5.2 Retention of clinical records

«Hospital Name» defines the retention period for each category of medical record — out-patient, in-patient, and medico-legal case — in consonance with rules laid down by the National Medical Commission and the respective state authority. This also applies to various data and the formats — for example registers and forms — used to capture that data.

5.3 Confidentiality and security of retention

The retention process at «Hospital Name» provides the same expected confidentiality and security described in other hospital policies, applicable to both manual and electronic systems.

5.4 Destruction

Destruction of medical records, data and information at «Hospital Name» takes place only after the retention period is over and after approval from the concerned authority, internal or external. The destruction procedure follows relevant statutory provisions.

6. Governance and responsibility

Role	Responsibility
Medical Records In-Charge	Owns this policy. Maintains the document-control system and retention/destruction schedules.
Document Owners / Department Heads	Author, review and authorise documents in their area; keep documents current.
Medical Superintendent	Oversees implementation of this policy. Approves record destruction requests.
All Staff	Use only current, released document versions. Report identified risks and rule breaches.

7. Quality monitoring

The Medical Records In-Charge reviews document-control compliance and retention-schedule adherence periodically. Documentary evidence is on file for each CORE objective element and each asterisked objective element (marked CORE and * respectively in the Level column of Section 12, Traceability table). This policy is reviewed annually, or sooner if NABH standards change.

8. Training and staff acknowledgement

All staff covered by this policy are trained on document-control procedures and how to access current, relevant documents for their work, and sign the acknowledgement below.

Acknowledgement statement:

"I have read the Policy on Availability and Retention of Current Documents, Records, Data and Information of «Hospital Name». I will follow the processes described."

Signature table (one row per staff member):

Staff Name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy is distributed to the Medical Records department, all department heads/unit in-charges, and is made available to all staff through the hospital intranet and department policy folders.

10. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
IMS	Information Management System (NABH Hospitals 6th Edition chapter)
OE	Objective Element

11. Traceability to NABH HCO Full Accreditation 6th Edition — IMS.6

This table is an index. It is not how the process is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
IMS.6.a	CORE*	Effective process for document control	Section 5.1	Medical Records In-Charge
IMS.6.b	CORE*	Clinical records retained per requirements	Section 5.2	Medical Records In-Charge
IMS.6.c	Commitment	Retention process confidential and secure	Section 5.3	Medical Records In-Charge
IMS.6.d	Commitment*	Destruction per written guidance	Section 5.4	Medical Superintendent

12. Required Records / Evidence Checklist

IMS.6.a — Document control.

- Document-creation/review/authorisation/release record naming designated individuals.
- Planned document-review-schedule record and obsolete-document-removal/archival record.

IMS.6.b — Retention of clinical records.

- Defined retention-period record per record category (out-patient, in-patient, medico-legal case), referencing National Medical Commission/state authority rules.
- Retention-period record for data-capture formats — registers, forms.

IMS.6.c — Confidentiality and security of retention.

- Retention-area access-control record, manual and electronic.

IMS.6.d — Destruction.

- Destruction-approval record from the concerned internal/external authority.
- Destruction-procedure record following applicable statutory provisions.

13. References

- NABH Guidebook for Accreditation of Hospitals, 6th Edition — Standard IMS.6, "The organisation ensures availability of current and relevant documents, records, data and information and provide for retention of the same."
- National Medical Commission (adopting the Indian Medical Council). (2002). Professional Conduct, Etiquette and Ethics Regulations (Section 1.3, Maintenance of Medical Records). <https://www.nmc.org.in/wp-content/uploads/2017/10/Ethics-Regulations-2002.pdf> — the regulation underlying the record-retention rules named in IMS.6's own text.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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